

Find the gaps you already closed.

Health plans pay abstractors to search 50-page charts for evidence of care that is already documented — and some of it is still missed. A skipped immunization note or a buried lab value counts as a gap failure, even though the care was delivered.

Martlet AI reads every page of every chart, points to the sentence holding the evidence, and pre-populates the measure fields — so your abstractors confirm findings instead of searching for them.

200%

abstractor
productivity

100%

of each
chart read

100k+

lives running
on the platform

Zero

PHI leaving
your network

What you actually get.

Runs on your own infrastructure

On-premise, private cloud, or air-gapped. **No chart shipping, no outsourced abstractors, no PHI leaving your network** — and every finding recorded for audit.

Reads every page

Every page of every chart goes through the same extraction, **including scanned and handwritten notes**. Nothing gets skipped when the deadline tightens.

Points to the exact sentence

Each finding comes back with **the sentence that supports it, the source document, the date, and the measure it applies to** — not a summary or a confidence score alone.

Abstractors confirm, not search

Measure fields arrive pre-populated with the evidence linked. **Your team reviews findings instead of hunting for them**. That is what doubles throughput.

From chart to measure status.

Clinical records go in. A status and its evidence come out, per member and per measure. **One system, with no hand-offs to an outsourced abstraction vendor.**

1

Ingest

EHR notes, labs, radiology, discharge summaries and scanned PDFs.

2

Extract

Medications, labs, vitals, procedures and screenings pulled from the text.

3

Map

Each finding matched to the measure it applies to, with its date.

4

Status

A status returned per member and measure, with the evidence behind it.

5

Submit

Abstractors confirm, and findings export in your submission format.

4

What comes back for each member and measure

Every result carries the evidence behind it — the source document, the date, and the sentence the finding came from. **Each one lands in one of three states.**

Closed

The record contains evidence that meets the measure. Fields are pre-populated for the abstractor to confirm.

Needs review

Evidence is present but incomplete, undated, or outside the measure window. Routed to an abstractor with the finding attached.

Open

Nothing in the record meets the measure. Passed to outreach or care management before the season closes.

Members with no gap in a measure are never queued for review — **abstractor time goes only to the records where a decision is needed.**

What gets pulled out of every chart.

Structured feeds rarely hold the whole picture. **These are read out of the record itself, including documents nobody has time to open.**

01 Medications

Name, dose and date, whether they sit in a structured feed or only in the note text.

03 Procedures and screenings

The procedure, the date it was performed, and the document it was found in.

05 Scanned and handwritten notes

Read with OCR and treated the same as typed text, rather than set aside.

02 Labs and vital signs

Values and their dates, including results that appear only inside a scanned report.

04 Immunizations

Including entries recorded in a note rather than an immunization record.

06 Dates and measure windows

Each finding checked against the window that applies to its own measure.

HEDIS specifications change every measurement year. We update the measure logic to match, so abstraction follows the specification in force.

What changes for your abstraction team.

The chart has already been read when they open the queue. **Their time goes to judgment calls, not to page-by-page searching.**

They confirm instead of search

Measure fields arrive filled in, with the supporting sentence linked to its page. **That is what doubles the number of charts an abstractor gets through.**

Every finding carries its evidence

Source document, page and date travel with each finding, so **the record supporting a closed gap is there** when it is asked for.

Volume stops setting the deadline

Chart volume can grow without adding abstractors or extending the season, because **the reading is done before your team starts.**

Abstractor overrides are tracked

Confirmation and override rates are tracked for each abstractor, so **you can see where they disagree with the engine** and where it needs tuning.

Why the found gaps matter.

HEDIS measures account for **25% of the CMS Star Rating for 2026**, and moving up a half Star can change quality bonus payments across the contract. Evidence sitting in a chart that nobody finds scores the same as care that was never delivered.

NCQA has published a timeline moving HEDIS to fully digital measures by measurement year 2030. Reading evidence directly out of the record is the direction the program is already heading.

In production at health systems, plans and platforms.



How the engagement works.

An annual license

You pay a license fee for the year. **Not per chart, and not per gap closed.**

Priced to your size

The fee is based on how many contracts and lives you run, and **stays fixed for the year.** Add volume without renegotiating.

Configured to your measures

We set the measure set, thresholds and exports to match how your team works, and **connect to the systems they already use.**

Bring one measure. We'll abstract it while you watch.

Charts ingested, evidence located and linked to the page, and a status returned for every member — inside your own environment.

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